ROTATION: SCHOOL HEALTH

Chapter 7 — School Health Rotation

Health services for the school-age child (6-12 years): the school nurse's roles, school entry appraisal, screening programs (vision, hearing, scoliosis, dental, weight status), immunization compliance, emergency care and first aid, in-school medication, and communicable-disease control.

Age range: 6 – 12 years

7.1 Site Profile — The School Health Program

School nurses assume a major role in the school health program and can significantly affect the lives of school-age children. Working collaboratively with the school and community, they provide health supervision, health counseling, and health education.

RESPONSIBILITIES OF THE SCHOOL NURSE

- Education and interventions for **acute and chronic illness, safety, health promotion, nutrition, mental health, dental disease, and sexually transmitted infections**.
- For some children, especially those in poverty, the school nurse may be the **only contact** with illness prevention and health promotion.
- For children with chronic conditions: leadership within the school health team, assessing health status, identifying barriers to learning, and developing an **individualized health care plan**.
- In school-based health centers, nurse practitioners provide primary care — treatment of acute illness, vision/hearing screening, **sports participation examinations**, nutrition counseling, and anticipatory guidance.
- Unlicensed assistive personnel (UAP) may be part of the team — state-certified paraprofessionals who assist a professional under supervision.

Figure 7.1 shows the coordinated school health program and the core services that sit inside its health-services component — appraisal, emergency care, immunization compliance, communicable-disease control, and referral.



Figure 7.1 The coordinated school health program and the core services of its health-services component.

7.2 School Entry Health Screening

Health appraisal at school entry covers screening tests (vision, hearing), measurements (height, weight), and medical, dental, and psychologic examinations.

Appraisal item	Content
History	Interval history (feeding, elimination, sleep, behavior, past illness, allergies, medications), immunization record; family health and psychosocial history
Measurements	Height and weight on CDC charts (2-20 yr); BMI for weight status — school-age obesity prevalence ~17%; blood pressure annually
Physical exam	Head-to-toe (eye acuity — Snellen letters for school-age; ear/hearing — pure-tone audiometry; spine — scoliosis screening §7.3; skin; teeth; abdomen; extremities)
Immunization review	Check the record against the printed schedule (see Ch. 4 §4.5); vaccines are required in most states before school entry

Tuberculosis / special screens	Follow the ministry/school regulatory screening requirements
Psychologic / social	Stress signals to explore: stomach pains/headache, sleep problems, bed-wetting, eating-habit changes, aggressive/stubborn behavior, reluctance to participate, regression; domestic-violence exposure risk awareness

7.3 Screening Programs

Screen	Method and referral
Vision	Distance acuity with Snellen letters (or numbers, tumbling E, HOTV) — referral: fewer than 4 of 6 correct on the 20-foot (6-m) line; alignment: corneal light reflex + cover test at near (33 cm) and far (6 m)
Hearing	Pure-tone audiometry at 0.5, 1, 2, and 4 kHz; fails if tones cannot be heard at 20 dB HL; tympanometry for middle-ear function
Scoliosis	With the child standing in underpants (bra for older girls) observe from behind — asymmetry of shoulders and hips; with forward bend (back parallel to the floor) observe from the side for rib-cage asymmetry/prominence; investigate a slight limp, crooked hemline, or sore back; inspect the spine for tufts of hair, dimples, or discoloration. Scoliosis is especially important in girls
Weight status / BMI	Plot height and weight on CDC charts; BMI for age; school-age obesity prevalence ~17%, higher in rural/socioeconomically distressed families; the school nurse takes an active role in nutrition
Dental	Brush 2-3×/day for 2-3 minutes with fluoridated toothpaste; floss at least once daily; soft toothbrush changed every 3-4 months; dental examinations every 6 months; dental sealants recommended for all school-age children

7.4 Immunization Compliance & Records

- **Before school entry:** immunizations are required in most states before entry, part of communicable-disease-control attendance policy.
- **At every contact:** verify the record, check live-vaccine timing, and arrange catch-up (Ch. 4 §4.5).

- **Routine boosters:** the booster ages for this age band follow the official printed schedule; the adolescent items (MenACWY, Tdap, HPV, annual influenza, HepB/HepA, varicella) are covered in Chapter 8.

7.5 Emergency Care & First Aid

- **Emergency care & safety:** emergency treatment (first aid), notification of parents, and transportation of the ill or injured child to home or hospital.
- **Chronic conditions:** develop an individualized health care plan (IHP) — the nurse assesses health status and identifies barriers to education; known allergy/asthma/seizure students follow their IHP and facility protocol.
- **Sports:** participation physical examinations at least **every 2 years**; the school nurse practitioner performs them in school-based centers.
- **First-aid training:** first-aid measures and child CPR are taught to parents; facility drills and kit contents follow school/ministry policy.
- **Injury-prevention baseline:** accidental injury remains the leading cause of death for children 1-14 years; motor-vehicle crashes are the most common cause of severe accidental injury/death for school-age children.

7.6 Medication Administration at School

SAFETY ANCHORS AND POLICY LABELS

- Medications must have **child-safety caps**; never leave any medication within a child's reach; "child-resistant" is not "child-proof".
- Children with chronic conditions receive care coordinated through the **individualized health care plan**.
- **UAP** assist a professional under supervision — who may administer, storage, dose verification, parental permission, and documentation follow the school's written policy and local law.

7.7 Health Education & Communicable Disease Control

- **Health education:** the school curriculum is a planned, sequential kindergarten-to-grade-12 program addressing physical, mental, emotional, and social dimensions of health.

- **Communicable-disease control:** detection and **exclusion** of affected children, with policies for readmission and attendance.
- **Nutrition services:** school meal programs furnish one third of the daily recommended allowance; the average school lunch may exceed guidelines for saturated and total fat — healthful choices (fruits, vegetables, low-fat milk) are reinforced.
- **Hand hygiene:** routine handwashing before and after every meal, toileting, and child contact.
- **Head lice, absentee surveillance, and reporting:** school policies for detection, exclusion/readmission, and disease reporting follow local regulations.

Chapter 7 Learning Check

Self-assessment questions (answer in your notebook, then open)

End of Chapter 7.